

Meeting Transcript — September 1, 2026

Participants: Thomas Abshier, Isak Gutierrez **Time:** ~6:26 AM – 8:38 AM **Platform:** Video call (Zoom/Meet)

1. Trading / Stocks (~6:26–6:37)

Thomas: It's kind of fun, but as far as being profitable — no, I don't think so. I mean, just a couple more days and I'd be down to zero.

Isak: Yeah. How do you justify all the effort?

Thomas: The effort and interest and attention... I think probably what would be reasonable, just for the fun of it — play one contract. Just call it something fun to do. We just let it ride. It doesn't matter one way or the other. We can get on with life. We just let it run and sell at the end of the day and just see how that system works. Because that is the system — buy it and let it go to the end of the day.

Isak: If we do that, what's the potential loss?

Thomas: Well, if you do one contract and it loses 10 points, you lose \$100. So it's not a big loss, not a big gain either. It's entertainment. It gets you up in the morning, gets you going. And we can actually just let it go, deal with business, and go on about our day.

[Market opens. Thomas enters a call at 4.396, sets closing order sale at 4.30. Isak enters at 4.347.]

Thomas: Made \$34. Hey — lose \$1,300, gain \$34. Well, I won something.

Isak: I got out at \$20.

Thomas: That's a lot more sane.

Isak: It is. It's less watching hundreds of dollars moving around.

Thomas: Yeah, we're playing penny poker instead. Let's put that down so we don't sit and spend our entire life watching random numbers change. It's completely addicting.

Isak: Heart racing, heart pulping.

Thomas: “Oh, I should have. Oh, I shouldn't have. Maybe I should get in now. Oh, I got in. Oh, I shouldn't have.” ... “Oh, I got in! Look — oh, I'm a genius! It's fantastic.”

Isak: “I'm so smart.” Let me grab my water really fast.

2. Joelle Hired (~6:28)

Thomas: Yesterday I hired Joelle. That's what she wanted to talk about — just getting paid. What that really brings up is: what do we actually have Joelle do?

I've been thinking about that a little bit, and the best thing I think could be done is her being your assistant. Kind of like you were trying to train Cayenne, and she wasn't able to really grasp it. But

you've got the big picture on what needs to be done, and Joelle is mature as a businesswoman. She can see what we're actually doing.

3. Charlie's Email — Two-Tier Product (~6:42–6:46)

Thomas: I got an email from your dad at 12:16. He was basically responding to the need to make two kinds of walkers.

Isak: (*reading email*) "Per our chat on the heavy-duty medical billing stuff and the need for at least two tiers of products. Basically: (1) LiftWalker does not disassemble for transport, pretty plain compared to what's possible, basic machine. Not discounted cheap seats, but not the deluxe model. Terrific construction and work on light quality. (2) Expensive model — same basic type but disassembles, pretty easy, some bells and whistles. Or the same as number two but includes every accessory including the suspension harness, thoracic prompts on the suspension walking harness. Two is amazingly beautiful, striking — prettiest colors, various nylon straps, carbon arm pads."

So he's talking about two versions of the LiftWalker — not the Traveler. Just the full frame.

Thomas: I think he's talking about basically making the Traveler but not be able to break it down — just a single assembled unit. The question is whether that's enough differentiation. My concern is getting audited and having it look like we're just trying to get around regulations.

Isak: The real concern would be that we're selling the same product for less somewhere else.

Thomas: Yes, that's my concern. I don't think this is gonna actually make the differentiation we need. Something that would make it actually different would be using a hydraulic lift instead of the motorized lift.

4. Making the LiftWalker Shippable (~6:46–7:00)

Thomas: Can we make it so that the new LiftWalker is disassemblable and the customer can assemble it? Both have the convenience of shipping, but once they're put together, they're not easy to take apart.

Isak: That's a maybe. With the way it's designed, there are welds they'd have to do. It's not really an assembly thing.

Thomas: So the question is: are the pieces that are welded small enough so we could put those in a box and have the parts that are not welded be bolted on?

Isak: I'll find out from Dad — what's the most disassembled version we can send, and is that reasonable? What will shipping cost be?

Thomas: There's a storage cost too. If we got 10 of these things assembled, we're going to need another storage room. If we can disassemble them and stack them and ship them disassembled, that would solve the whole shipping and storage problem.

[Thomas draws diagrams on whiteboard explaining a rod clamp system for connecting the cross-bar, center strut, and rocker arm — a one-time assembly connection using rod clamps, plates, and pin connections rather than welding.]

Thomas: Some assembly required. You're going to do it once and it's assembled.

Isak: Okay. So how is this different from the Traveler?

Thomas: This is a permanent assembly versus a disassembly. It's a new LiftWalker that can be disassembled and put together once. You're not going to do this more than once.

5. Product Strategy — LiftWalker vs Traveler (~7:15–7:20)

Isak: How do we sell the new LiftWalker versus the Traveler?

Thomas: I've always held the new LiftWalker as personal to clinics. We sell the new LiftWalker to clinics. We sell the Traveler to individuals for home use.

Once we establish a relationship with a clinic, we can talk to them — we have these units at \$7,000 apiece. If they want to sell a Traveler to a patient for home use, they can do the prescription thing.

It's actually to the advantage of the clinic to NOT have people have the Traveler available to them, because then they won't be coming to their clinics.

Isak: That's true.

Thomas: Which actually reduces the need for the second version. We're actually just trying to get people to have their own unit. They could say "you buy it, you bring it here, and we'll do therapy with you." So everybody is incentivized to have one. Having one simply at a clinic is actually not doing the client a favor — they're saying "once a week you'll get a chance to get on your feet."

The people that want this for institutional use are people like DADD that have a group home. They need to move people around. That's an entirely different market.

6. Walker vs Transfer Device — Regulatory Discussion (~7:27–7:42)

Thomas: The transfer, the out-of-bed, the toileting — those are the big ones. Getting from a wheelchair to the toilet seems like the kind of thing that would hurt the helper.

[Thomas shares personal story about his father hurting his neck while helping his grandfather transfer from a couch.]

Thomas: This is a transfer device. Get them out of bed in the morning. That's the way I've always seen this unit — something that really helps people transfer.

Isak: *(reading from competitor analysis)* For the FDA classification: patient lift and transfer device, FSA — significantly heavier. Full quality system, 20 to 35 documented procedures, CAPA, supplier controls, internal audits, design controls. Still Class I and 510(k) exempt, but GMP requirements alone are a major operational burden for a small company.

The Traveler straddles both categories — it's a walker and it lifts people. Classification hinges on intended use statement. Rifton splits their line: manual Pacers are ITJ (walker), powered E-Pacer and TRAM are FSA. Biodex found a third path — registered the Mobility Assist under BXB (powered exerciser), avoiding FSA entirely for a functionally similar device.

It really is about what we call it. If we're ready to jump into the whole patient lift industry... I mean, we could make some incredible patient lifts, but we're just talking about this product.

It doesn't really transfer to the couch unless you have a couch with room underneath for the legs and wheels. It doesn't really drop you far back enough. It's not perfect for all transfer things.

Thomas: Let's say the one thing we want to transfer them to is the toilet. That's the one thing everybody needs and everybody has trouble with. So that's all we talk about — we're transferring to the toilet.

Isak: My point is it's not really adequate if it's sold as a transfer device for all things. If we were to make it a designated transfer device, it would look different versus a walker.

7. Competitor Analysis Review (~7:50–8:21)

Thomas: What is it that Rifton, Biodex, and the others — what's their angle on making their product desirable?

Isak: *(reading from Claude Code analysis)*

Rifton: Designed for dignity. Leads with human stories, independence, inclusion. Strong pediatric focus. Only company covering pediatric to adult with one product family. Bluetooth scale with mobile app for tracking weight-bearing progress. US-made. Heavy educational content, therapist video testimonials, insurance navigation help, free shipping. No e-commerce — phone/email sales.

Biodex: Biomechanical precision and clinical data — the “science company.” Facilities only. Patented 4-bar and 6-bar linkage systems that emulate natural sit-to-stand kinematics. 70+ years of clinical brand credibility. Targets Safe Patient Handling laws in 11 states as market driver. Price ~\$5,900.

Mobility Research (HugN-Go / LiteGait): 3-in-1 home viability — ambulation, sit-to-stand, and seated transfers in one compact device. Both facilities and home users. Small enough for home doorways and bathrooms. One-caregiver operation. Rechargeable battery. Price quote-gated, estimated \$7,000–\$12,000.

Arjo (Hi-Lo Walker): Outcomes-based ecosystem selling — not just a device but a full Safe Patient Handling & Mobility program. Enterprise facilities exclusively. Mobility Gallery — a 5-level patient assessment framework. Facilities adopt the framework, then need Arjo equipment to implement it. Consultative lock-in strategy. Price \$6,000–\$12,000.

Bure (Rise & Go): Cost-effective 3-in-1 — replaces separate hoists and walkers. Facilities primarily. Patented power rise. Electrically driven frame widening (26”–42”) to fit around beds and wheelchairs — unique feature nobody else has. Swedish hospital clinical validation. Price \$7,000–\$9,000.

Key insight from analysis: Rifton's content marketing at industrial scale. Education center holds sample letters of medical necessity, funding guide workbooks, evaluating forms, competency checklists, in-service videos, customer story roundups, and a newsletter. Their Pacer LMN page explicitly explains that each unique feature must be separately justified as medically necessary.

Arjo: Trademarked both "Positive Eight" and "Mobility Gallery." The Mobility Gallery sorts residents into levels A–E with names and personas, and every suitable brochure states which persona it serves. The Sara Flex brochure literally says "suitable for Carl." The effect is the sales conversation stops being "do you want a lift?" and becomes "what is your resident mix and where are the gaps?"

Mobility vs transfer — the real split: Transfer products are sold on caregiver injury and liability. The buyer is a risk manager or facility administrator. The argument is workers' comp, OSHA zero-lift policy, staff injury rates — a cost-avoidance calculation, not a clinical one. Mobility and gait products are sold on patient outcome and funding justification. The buyer is the therapist acting as prescriber with a payer or IEP behind them.

Biodex shows you can take the caregiver safety benefit without taking the transfer device identity. Their copy says "no patient lifting required, mitigates injury to staff, reduces risk of falls" — while the product is still named "Mobility Assist" and described as a stand assist that leads into ambulation. That is precisely the needle we need to thread.

Rifton collapsed their transfer device (TRAM) into their gait trainer line (E-Pacer) in June 2025. They moved toward our positioning, not away from it.

8. Sales Challenge & Pricing Discussion (~8:21–8:38)

Thomas: The thing that's striking is we're charging \$16,000 and other people are charging \$6,000. That puts us out of any competitive market. Doctors are probably going to see the difference and think "I can't in good conscience prescribe this expensive thing when a cheaper one is available."

We could do another strategy — charge the same as everybody else, make less per unit, but make it up in scale because we've got everybody prescribing it.

Isak: What does that price look like to you?

Thomas: It sounds like people are pitching \$6,000 to \$12,000. The thing they hear is \$6,000 — that's the problem.

[They work through DME economics:] - At \$6,000 MSRP: Medicaid pays 75% = \$4,500. Minus DME margin → down to unit cost. **Not functional.** - At \$12,000 MSRP: Medicaid pays 75% = \$9,000. Minus DME 30% (\$2,700) = \$6,300 to Ideomotion. Minus COGS (\$3,500) = **~\$2,300 profit per unit.**

Thomas: We could literally do 2 tracks. Try the \$16,000 prescription/DME track AND sell direct for \$7,000. Two different products — one goes through the public system, the other you pay for directly.

Isak: How do we differentiate the models? If we're worried about an audit...

Thomas: They're just two different products. They're not the same. And that's defensible. We have no track record. Your dad sold them at ones and twos — high service, high touch. And it

burned him out. It's hard.

Thomas: When I got into this project, I didn't know any of this. I just heard your dad's enthusiasm — “we got a better product.” And I said okay, let's do it. That was the level of market research I did.

We've spent a lot of money and time developing a product which is already a mature product available from competitors. The most obvious thing to do is keep doing what your dad has already done — sell to schools, sell to individuals, sell to institutions. Try and sell it as a therapy device. The possibility of actually walking as opposed to just sitting.

We're really talking about creating a new medical paradigm — getting people to believe that someone in a wheelchair could actually learn to walk. Nobody even thinks that. You see a person in a wheelchair, you go “that person can't walk.” I would never think “if they were able to get up and practice walking, they would be able to walk.”

These competitors are well established. They've already done the homework — documentation, supply pipelines, advertising, staffing. We've got none of it. We have a dream.